

Daily Content Intel

May 9, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, here's a clinical move I use almost every week with athletes who can't take time off, but their tendon is screaming.

Patellar tendon pain in-season is the classic problem. The athlete pushes through, the pain gets louder, they start losing power, and the season won't pause for a rest week.

The research here is pretty clear. Heavy isometric holds drop tendon pain in the short term, and the analgesia lasts. A recent systematic review found the pain reduction was sustained at 45 minutes post-exercise. So the athlete warms up with isometrics, walks into practice with a quieter tendon, and actually trains.

The protocol I run is 5 sets of 45-second holds at about 70% of max voluntary contraction. Quiet on a leg extension or a Spanish squat against a band. Aim for tension during the hold. Pain should sit around 2-3/10 the whole time.

The longer-term piece is heavy slow resistance, 3 days a week, slow tempo, working through a rep range that climbs over months. That's where the tendon actually remodels.

In-season, isometrics buy you the day. Off-season, heavy slow resistance builds the tendon. Both have a job.

Be Good. Help Someone. Learn Lots.

Hook: Patellar tendon pain in-season? You can drop it 30 minutes from now without resting.

Spoken script (word for word):

Okay, here's a clinical move I use almost every week with athletes who can't take time off, but their tendon is screaming.

Patellar tendon pain in-season is the classic problem. The athlete pushes through, the pain gets louder, they start losing power, and the season won't pause for a rest week.

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On-screen text seeds:

- 0:00 "Tendon pain in-season? Here's the move."
- 0:08 "Heavy isometric holds. Short-term analgesia. Up to 45 min."
- 0:25 "5 x 45 sec @ 70% MVC"
- 0:35 "Pain target during hold: 2-3/10"
- 0:50 "Long game: heavy slow resistance, 3x/week"
- 1:05 "Threshold Health & Performance"

Caption (copy-paste ready):

Patellar tendon pain in-season is one of the most common complaints I get. There's a clinical move that drops the pain inside the same session.

Heavy isometric holds give you a 45-minute window of quieter pain that lets the athlete train. 5 sets of 45 seconds, around 70% of max voluntary contraction, pain sitting at 2-3/10 the whole time.

Heavy slow resistance is the long-term remodeling tool. 3 days a week, slow tempo, climbing rep ranges over months.

Both have a job. The trick is knowing which one to pull on which day.

Your threshold is higher than they told you.

Dr. Lars Stevenson, PT, DPT threshold.clientsecure.me

#patellartendinopathy #physicaltherapy #tendinopathy #isometrics
#sportsmedicine

Personalize this

- What's a recent in-season patellar tendinopathy case where I used the isometric warm-up trick before practice and watched the athlete's quality of work change inside the same session? Name the sport and the position.
- How does the CROSS framework's "Reset" pillar specifically map to this isometric loading protocol on day-of, and how does "Strengthen" map to the heavy slow resistance phase later in the rehab?
- What's the specific Spanish squat or leg-extension setup I prefer in the clinic, and is there a tweak I make for athletes who can't tolerate the open chain leg extension?

Key passage:

"Isometric exercise leads in significantly reduced tendon discomfort instantly and the reduction in pain was sustained when assessed at 45 min post exercise."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC12259690/>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, hot take time, and I want to pick a fight with the rehab world for a minute.

Decline-board eccentrics for patellar tendinopathy. That protocol came out in the early 2000s and it's still the first thing a lot of clinicians prescribe. 20 plus years later, the meta-analyses are saying something different.

A 2024 network meta-analysis on patellar tendinopathy compared eccentric, isometric, and heavy slow resistance head to head on the VISA-P score. The line that mattered, and I'm quoting, "the PTLE that includes isometric training or moderate slow resistance is more effective than eccentric training alone."

PTLE means progressive tendon loading exercise. Isometrics for the acute pain. Heavy slow resistance for the long-term adaptation. Eccentrics still have a role, but they shouldn't carry the whole program, and they probably shouldn't be the first tool out of the box.

Here's what bugs me. The decline-squat eccentric is uncomfortable, drives soreness, and most athletes hate it. The field had the option to layer in isometrics for symptom relief and heavier compound lifts for slow remodeling, and a lot of clinics stuck with the old protocol because it was already in the EHR template.

If you've been doing decline eccentrics for 6 weeks and your knee still hurts, the program is the problem.

Be Good. Help Someone. Learn Lots.

Hook: If your PT is still prescribing only eccentrics for your patellar tendon, they're working off a 2003 playbook.

Spoken script (word for word):

Okay, hot take time, and I want to pick a fight with the rehab world for a minute.

Decline-board eccentrics for patellar tendinopathy. That protocol came out in the early 2000s and it's still the first thing a lot of clinicians prescribe. 20 plus years later, the meta-analyses are saying something different.

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On-screen text seeds:

- 0:00 "Eccentrics-only for tendon pain = 2003 playbook"
- 0:15 "2024 meta-analysis: PTLE > eccentric alone"
- 0:30 "Isometrics for acute pain"
- 0:38 "HSR for long-term adaptation"
- 0:55 "6 weeks on decline eccentrics, still hurts? Program problem."
- 1:08 "Threshold Health & Performance"

Caption (copy-paste ready):

The decline-board eccentric for patellar tendinopathy turned 20 last year. It's still the first thing a lot of athletes get handed.

A 2024 network meta-analysis lined up eccentric, isometric, and heavy slow resistance and the verdict was clean: progressive tendon loading that layers in isometrics or moderate slow resistance beats eccentric work done in isolation.

Isometrics drop the in-session pain. Heavy slow resistance does the long-term tendon adaptation. Eccentrics still have a role inside a bigger program. They shouldn't be the program.

If you've been grinding decline eccentrics for 6 weeks and the knee still talks to you, ask your PT what the rest of the plan looks like.

Your threshold is higher than they told you.

Dr. Lars Stevenson, PT, DPT threshold.clientsecure.me

#patellartendinopathy #sportsphysicaltherapy #tendinopathy #rehab
#thresholdhealth

Personalize this

- Which patient (volleyball, basketball, or jumping athlete) had been doing decline eccentrics from a previous provider, came in plateaued, and got better once I layered in isometrics and heavy slow resistance? Name the in-clinic outcome that proved it.
- What's the specific reason eccentrics were the right call in 2003 (Alfredson Achilles work, in-season volleyball cohorts), and where does the science genuinely stand on them now inside a layered program?
- How does this fit the CROSS framework's "Strengthen" pillar specifically, and how do I explain the difference between symptom-day work and adaptation-day work to an athlete during the eval?

Key passage:

"The PTLE that includes isometric training or moderate slow resistance is more effective than eccentric training alone."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC11570476/>

Reference

Runner-up topics

- JSCR May 2026 foot march meta-analysis (lower-body strength, power, postural stability decrements after load carriage). Full text was paywalled today, retry through library proxy or wait for an open-access summary.
- Peter Attia AMA #84 minimum effective dose strength training takeaways once a transcript or open summary surfaces.
- IJSPT "Oh, My Quad" framework for ACL rehab quad recovery, paired with limb symmetry index thresholds at 7 and 9 months post-op.

Sources scanned today

- Newsletter digest folder: latest file is 2026-04-02, more than 5 weeks stale, skipped.
- PubMed recent rehab/PT/sports med queries via WebSearch.
- JSCR May 2026 issue (foot march meta-analysis primary article paywalled, 402 from full-text fetch).
- Peter Attia Drive AMA #84 (May 4, 2026) — show notes available, full content gated behind premium.
- Huberman Lab — 2 May episodes (Halpern DBS, Conti mental health), no clinically actionable PT angle.
- IJSPT and Heliyon network meta-analyses on tendinopathy loading protocols (open access, used as primary).
- NSCA articles, Pacey Performance, r/AdvancedFitness — checked, no fresh week-old hooks beat the tendinopathy meta-analysis.
- Earlier today's run already covered 2-min HRR test (Draft A) and ACL clearance criteria (Draft B). Today's drafts pivot to tendinopathy loading to keep URLs distinct from the registry.